



CORPORATE CAPABILITY & BUSINESS DEVELOPMENT

**We do not write reports
about African healthcare.
We run it.**

Nine service lines. Eight engagement models. Three proprietary platforms.
Healthcare transformation and management across Africa.

ORIENTATION

The firm in one page

An Africa-focused healthcare management and transformation firm. Built by operators, for operators.

135+ yrs

Combined senior healthcare leadership across the partner network

20+

Senior operators in the C4A network, fielded as integrated teams

16

Professional cadres covered by our workforce platform

3

Proprietary platforms owned and operated by the firm

What we are	A management and transformation firm, not an advisory house. We embed, implement, and stay long enough to hand over something that works.
Who we serve	Private hospitals and groups, payers and HMOs, investors and DFIs, governments and development partners, NGOs, life sciences, and healthcare ventures.
Where we operate	Headquartered in Lagos and Abuja, delivering across Nigeria and into Ghana, Kenya, and the wider continent.
How we are paid	Fixed-scope projects, monthly retainers, secondment day rates, fractional mandates, and equity-linked turnaround structures.



THE PREMISE

Africa's health systems do not have a knowledge problem

They have an execution problem. That is the whole thesis of this firm.

The strategy exists

Boards have commissioned the reports. The recommendations are usually right. Almost none of them have been implemented.

The capability is thin

Senior leaders were promoted on clinical tenure, not on management capability, and nobody has ever measured the difference.

The workforce is leaving

Every vacancy costs months of locum premium and referral leakage while the post sits open.

Nobody owns the number

Revenue leaks, theatre lists run half empty, and marketing spend disappears, because no single person is accountable for the outcome.

Every service line in this deck exists to close one of those four gaps.

POSITIONING

What separates us from a consulting firm

The difference is not the analysis. It is who is standing in the building when the plan has to work.

THE TRADITIONAL MODEL	THE C4A MODEL
Diagnoses, then leaves	Diagnoses, then stays to deliver
Partners sell, juniors deliver	Former hospital CEOs and clinical leaders on the ground
Frameworks imported from other markets	Models built for African cost, payer, and workforce realities
Success measured by the report	Success measured by revenue, utilisation, and governance
Dependency by design	Capability transferred so performance holds after we leave



WHAT WE DO

Nine service lines, one integrated team

Take one. Take several. Most institutions start with one and widen the mandate once it works.

01

Hospital Turnaround & Financial Recovery

REVENUE CAPTURE

02

Strategy, Growth & Commercial Performance

SERVICE LINE STRATEGY

03

Clinical Governance & Accreditation

GOVERNANCE FRAMEWORKS

04

Digital Health & Technology Leadership

OPERATING INTELLIGENCE

05

Fractional Leadership & Executive Secondments

FRACTIONAL C-SUITE

06

Health Systems & Public Sector Advisory

SYSTEM DESIGN

07

Healthcare HR Management

RETENTION STRATEGY

08

Healthcare Workforce Recruitment

PERMANENT RECRUITMENT

09

Healthcare Marketing Agency

NEW PRACTICE

SERVICE LINE 01

Hospital Turnaround & Financial Recovery

When cashflow tightens and instability sets in, hospitals need decisive action. We restore financial control, stop revenue leakage, and rebuild operational discipline.

Revenue capture

Billing integrity, claims recovery, payer reconciliation, and price realisation.

Cost discipline

Procurement control, stock and consumable leakage, staffing cost to revenue.

Productivity

Theatre utilisation, bed days, clinic throughput, and diagnostics yield.

Cash and control

Cashflow stabilisation, financial visibility, and a management reporting cadence.

TYPICAL MANDATE

Equity-backed turnaround or fixed-term management mandate, 24 to 60 months

BEST FOR

Distressed or underperforming facilities, and lenders or investors holding the asset



SERVICE LINE 02

Strategy, Growth & Commercial Performance

Growth comes from aligning clinical strengths with demand, referral flows, and patient access. Not from adding more services and hoping for the best.

Service line strategy

Where demand actually is, what it is worth, and what you are equipped to win.

Referral networks

Physician engagement, partner clinics, and closing referral leakage.

Payer and pricing

Payer mix, HMO dependency, tariff negotiation, and self-pay positioning.

Diversification

New revenue lines, patient experience, and retention economics.

TYPICAL MANDATE

Advisory project, 8 to 16 weeks, with an optional execution phase to month 14

BEST FOR

Groups expanding into new markets and facilities with underperforming service lines



SERVICE LINE 03

Clinical Governance & Accreditation

Strong quality systems protect patients and build institutional credibility. We strengthen governance structures and prepare institutions for external accreditation.

Governance frameworks

Committee structure, escalation, clinical accountability, and board reporting.

Accreditation readiness

JCI, COHSASA, and SafeCare preparation, gap closure, and mock survey.

Patient safety

Incident systems, mortality and morbidity review, and risk registers.

Clinical audit

Audit cycles, quality monitoring, and clinical KPI reporting.

TYPICAL MANDATE

Diagnostic audit then a 6 to 18 month readiness and implementation programme

BEST FOR

Boards facing quality or safety exposure, and facilities pursuing accreditation



SERVICE LINE 04

Digital Health & Technology Leadership

We help healthcare organisations and ventures get the technology foundation right, from system selection to digital strategy to the teams that actually run it.

Operating intelligence

Executive dashboards, KPI trees, and decision-grade reporting.

Systems

HIS and EMR selection, implementation oversight, and workflow digitisation.

CTO as a service

Technical leadership for healthtech ventures without a full-time hire.

Commercial flexibility

Option to convert CTO fees into pre-seed equity.

TYPICAL MANDATE

Fractional or project mandate, 6 to 18 months, on-site or hybrid

BEST FOR

Hospitals digitising under pressure and healthtech ventures pre-Series A



SERVICE LINE 05

Fractional Leadership & Executive Secondments

Not every institution needs a full-time C-suite hire. We embed experienced healthcare executives on fixed-term mandates to fill leadership gaps and drive specific transformations.

Fractional C-suite

CEO, COO, CMO, and CTO on 2 to 3 days a week, on-site or hybrid.

Interim leadership

Hospital Director and Medical Director cover through transitions.

Embedded operators

Project-embedded clinical and operational leads inside your team.

Terms

Performance-linked mandates with an optional conversion to permanent hire.

TYPICAL MANDATE

Secondment 3 to 12 months, fractional leadership 6 to 18 months

BEST FOR

Institutions between leaders, in transition, or carrying a capability gap they cannot hire for



SERVICE LINE 06

Health Systems & Public Sector Advisory

We support governments, development partners, and NGOs on health system design, hospital network planning, and moving policy into implementation.

System design

Service configuration, hospital network planning, and referral architecture.

Policy to delivery

Turning approved policy into funded, sequenced, and monitored programmes.

Primary care

PHC strengthening, facility readiness, and workforce distribution.

Partner advisory

Programme design, delivery oversight, and results reporting for funders.

TYPICAL MANDATE

Programme mandate, typically 6 to 24 months, milestone-based

BEST FOR

Ministries and state governments, DFIs, development partners, and NGOs

SERVICE LINE 07

Healthcare HR Management

The workforce crisis is the single biggest threat to African healthcare. We combine consulting with proprietary assessment technology to help institutions hire, develop, and retain clinical leaders.

Retention strategy

Why your clinicians leave, what it costs, and what actually changes it.

Executive search

Senior appointments supported by Maarova™ leadership assessment.

Leadership development

Coaching, succession planning, and capability building for clinical leaders.

Workforce planning

Establishment review, compensation design, and culture measurement.

TYPICAL MANDATE

Project or retained, powered by the Maarova™ assessment platform

BEST FOR

Institutions losing senior clinicians and boards appointing into critical roles



SERVICE LINE 08

Healthcare Workforce Recruitment

Access a verified pool of healthcare professionals across 16 cadres. Permanent hires, locum cover, and structured pre-employment assessment, run on our own platform.

Permanent recruitment

Clinical and non-clinical roles sourced from a verified talent pool.

Locum and surge

Temporary staffing filled in days, not months.

Verification

Credential checks and structured pre-employment assessment.

Market intelligence

Salary benchmarking and workforce analytics by cadre and state.

TYPICAL MANDATE

Ongoing, per-placement or retained, delivered through CadreHealth

BEST FOR

Any facility carrying vacancies, and groups standardising how they hire



SERVICE LINE 09

Healthcare Marketing Agency

Empowering health brands through strategic marketing. A specialist agency inside the firm, blending data-driven strategy with medical expertise and creativity.

Brand and positioning

Brand architecture, value proposition, segmentation, identity, and guidelines.

Research and insight

Epidemiological and competitive analysis, journey mapping, health equity audits.

Campaigns and creative

Multichannel planning, medically vetted content, video, and influencer programmes.

Digital and analytics

Web and app build, SEO, social, CRM, and live dashboards measuring ROI.

TYPICAL MANDATE

Retainer advisory, fixed-price project, or a hybrid of the two

BEST FOR

Hospitals, HMOs, NGOs, and life sciences companies building brands and moving behaviour

SPOTLIGHT: NEW PRACTICE

Inside the marketing agency

We are more than an agency. We are healthcare partners. Behavioural science, digital analytics, and human-centred design, applied to health brands.

01

Discovery & Insight

Market research and stakeholder interviews with patients, clinicians, and regulators. Channel, competitor, and epidemiological audit.

02

Strategy & Planning

Positioning, messaging hierarchy, media strategy, and KPI framework, aligned to business and clinical objectives.

03

Creative & Campaign

Branding, storytelling, and digital content that simplify complexity without losing scientific integrity. Medically vetted.

04

Execute & Optimise

Multichannel deployment with compliance checks, live dashboards, behavioural journey design, and measured ROI.

SERVICE OFFERINGS

Brand strategy & positioning

Market research & insights

Campaign planning & management

Creative services

Digital marketing & technology

Influencer & community engagement

Public relations & advertising

Analytics & CRM

Engagement: retainer advisory, fixed-price project, or a hybrid of the two. Serving hospitals, HMOs, NGOs, and life sciences companies.



PROPRIETARY PLATFORMS

We own the infrastructure, not just the advice

Platforms built by the firm and used inside client engagements, or licensed directly.

Maarova™

Leadership assessment & development

- Psychometric assessment built for African healthcare leadership
- 360 feedback, development reporting, and executive coaching
- Used in executive search, succession, and appointment decisions

CadreHealth

Healthcare workforce platform

- Verified professionals across 16 cadres and 73 hospitals
- Permanent and locum recruitment, credential verification
- Salary intelligence and workforce benchmarking data

Agent Channel

Commercial distribution network

- Commission-based sales agents recruited from our clinical network
- Territory management, deal tracking, and attribution
- Route to market for healthcare products and services

Clients reach these platforms through an engagement, or licence them directly.

ENGAGEMENT MODELS

Eight ways to work with us

Every institution has different constraints. The commercial structure flexes, the accountability does not.

Advisory project Fixed scope, defined deliverables, embedded team 8 to 16 weeks	Retainer advisory Monthly hours pool with priority specialist access 6 to 12 months rolling
Embedded secondment Full-time consultant reporting into your leadership 3 to 12 months	Fractional leadership Part-time C-suite, 2 to 3 days a week 6 to 18 months
Hospital transformation Equity-backed turnaround with management control 24 to 60 months	Transaction advisory M&A and capital raising for healthcare assets 6 to 18 months
Healthcare recruitment Permanent, locum, and assessment via CadreHealth Ongoing	Commercial distribution Managed commission agent network, Agent Channel Ongoing



DELIVERY METHOD

The C4A execution model

Five stages. The last one is the one most firms skip, and it is the reason performance holds.

01**Diagnose**

Deep operational and financial assessment. We find the real problem, not the presenting one.

02**Design**

A bespoke plan with clear milestones, ownership, and measurable targets.

03**Deploy**

Embedded execution. Our team works inside the institution alongside your people.

04**Deliver**

Measurable outcomes: revenue recovered, costs reduced, governance strengthened.

05**Transfer**

Capability and systems handed to your team so performance sustains after we leave.

We report on a fixed cadence throughout, to management monthly and to the board quarterly.

WHAT WE MOVE

Where performance improves, value follows

Four levers carry most of the value in a healthcare institution. We are measured on all four.

Revenue recovery Capture earned income, close billing gaps, and fix payer reconciliation	Utilisation gains Theatre lists, bed days, clinic slots, and diagnostics running at capacity	Cost discipline Procurement leakage, consumable waste, and staffing cost to revenue	Governance strength Board oversight, clinical accountability, and decision-grade reporting
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TRACK RECORD

\$1.1M+ Annual savings delivered in a single engagement	135+ yrs Combined senior leadership across the partner network	20+ Senior operators fielded as integrated teams	16 Professional cadres in the workforce platform
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“Their embedded execution approach restored operational discipline, strengthened governance, and rebuilt leadership accountability.” Hospital board representative



CLIENT SEGMENTS

Who we serve, and what they come to us for

Private hospitals and groups

- Turnaround, growth, governance, workforce, and marketing

Payers, HMOs and insurers

- Provider network strategy, member enrolment, engagement, and cost control

Investors, PE and DFIs

- Commercial and operational due diligence, then post-deal value creation

Governments and ministries

- Health system design, network planning, and policy to delivery

NGOs and development partners

- Programme design, behaviour change campaigns, and results reporting

Life sciences and pharma

- Market entry, brand launch, and medically vetted commercial campaigns

Healthcare ventures

- CTO as a service, go to market, and distribution through Agent Channel

Diaspora-owned institutions

- Market entry, build and operate mandates, and clinical governance

OUR PEOPLE

Who actually shows up

Fielded as an integrated team, not as individual consultants billing hours.

Hospital CEOs and executive operators

Founders and former CEOs of Nigerian and East African hospital groups, with real P&L accountability behind them.

Clinical governance authorities

Clinicians who have led accreditation, quality systems, and patient safety programmes at scale.

Finance and revenue strategists

Specialists in billing integrity, NHIS and HMO strategy, payer negotiation, and cost reduction.

Marketing, brand and digital specialists

A multidisciplinary agency team spanning strategy, creative, technology, and analytics, vetted by clinicians.

Led by Dr Debo Odulana, Founding Partner. Former CEO, Cedarcrest Hospitals Abuja. Former Chief Innovation & Strategy Officer, Evercare Hospital Lekki. Founder of Doctoora.

COMMERCIAL PATHWAY

How an engagement starts

Low commitment at the front. The scope widens only once something has been proven.

STEP 01 Discovery call 30 minutes, confidential, no charge	We establish the presenting problem, the constraints, and whether we are the right firm for it.
STEP 02 Diagnostic 2 to 4 weeks, fixed fee	A focused review of the operational, commercial, or brand position, delivered as a board-ready report with a prioritised fix list.
STEP 03 Scoped mandate Structure follows the finding	Project, retainer, secondment, fractional leadership, or a full transformation mandate. Priced against the value at stake.
STEP 04 Delivery and transfer Monthly and quarterly reporting	Embedded execution against agreed targets, then handover of systems and capability to your team.



START THE CONVERSATION

Tell us what is not working.
We will tell you whether we can fix it.

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OFFICES

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